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Adrabetadex Provider Economics and Adoption Risk

Why Net Cost Recovery Determines Use

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Purpose

The calculator converts coding, payment, payer controls, acquisition cost, and claim timing into a provider-level go / no-go adoption signal.

Providers adopt when treatment is financially viable, not only when clinically convinced.

1. The Core Problem: Provider Hesitation

Issue	Provider concern	Impact on adoption
Acquisition cost	Provider buys high-cost drug before payment	Working-capital exposure
Coding pathway	No product-specific code at launch	Manual claims and denial risk
Payment timing	30-90+ day lag	Cash tied up across treatment cycles
Denials and rework	Prior authorization and documentation burden	Admin cost and revenue leakage
Site controls	White-bagging or site-of-care limits	Drug margin or ability to treat may disappear

2. Coding Pathway: Payment Becomes Routine Only After the Billing Path Stabilizes



3. Pass-Through Status: Temporary Stabilizer, Not a Permanent Answer

Scenario	Payment structure	Provider risk
No pass-through	Drug may be bundled or manually priced	High margin uncertainty
Pass-through active	Temporary separate OPPS payment	Improves early payment visibility
After expiration	Economics rely on coding and contracting	Long-term viability must be proven

4. EAP vs Commercial: Early Experience Can Overstate Launch Readiness

Dimension	EAP	Commercial buy-and-bill
Drug cost	Often free or subsidized	Provider purchases at WAC or 340B
Billing work	Minimal	Full coding and claims burden
Financial risk	Low	High until paid
Adoption read	Can overstate readiness	Shows real launch viability

5. Payer Friction: Policy Rules Directly Shape Provider Behavior



Prior auth

Approval before treatment. Delays care and weakens claim certainty.



White-bagging

Payer pharmacy ships drug. Provider loses drug spread and control.



Site-of-care

Payer steers setting. Hospital economics may not hold.

6. 340B Impact: Acquisition Advantage Can Determine Early Site Activation

Scenario	Acquisition	Reimbursement	Margin signal
Non-340B	~\$39,000	ASP/WAC-based	Thin or negative after delays and denials
340B COE	~\$30,000 planning assumption	Similar benchmark	Stronger per-dose margin runway
White-bagged	No provider purchase	Specialty pharmacy bills drug	No drug margin; admin economics matter

7. Calculator Value: Inputs Become a Provider Go / No-Go Signal

Inputs

Code, pass-through, site, payer mix, 340B, channel, denials, timing.



Model

Expected net provider position after payment, cost, denial, rework, and carrying cost.



Outputs

Margin, exposure, cash flow, break-even support, viability.

8. Plain-English Terms

HCPCS

Billing code set used for drugs and services; a J-code makes product billing more routine.

Pass-through

Temporary OPPS separate payment for certain new outpatient drugs and biologicals.

Buy-and-bill

Provider buys the drug, administers it, then bills the payer.

340B

Federal program allowing eligible hospitals to acquire outpatient drugs at discounted prices.

Providers adopt when financially viable, not only when clinically convinced.

The calculator turns that viability test into a launch-planning and access-strategy tool.